

Otras Causas De Disfonías Aguda

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Dx: Específico • Tx: Completo • Seg: Completo

Enfoque de Heridas Faciales y Cefálicas EUNACOM

1. Heridas de Cara (El Privilegio de la Irrigación)

Los traumatismos percutáneos faciales poseen una abrumadora protección inmunológica natural debido a su vascularización intensiva. Las heridas faciales raramente se infectan, pero **toda laceración de cara arrastra el riesgo médico-legal y psicosocial ESTÉTICO.**

• El ABC Quirúrgico Menor:

- La clave del aseo quirúrgico es la "Irrigación a presión Profusa". Uso irrestricto de ampollas de suero fisiológico y aguja/jeringa grande antes de afrontar, más el peinado macroscópico con pinzas para barrer incrustaciones y detritos.
- El afrontamiento (sutura) de elección de medicina general para la cara requiere sedales/nylon biológicamente inertes e indelebiles (Monofilamento), idealmente asilando técnicas de **sutura INTRADÉRMICA**. (Evitar puntos sueltos transperforantes asintomáticos aletargadores o corcheter cicatrices queloides horribles letales de asombro orales fofitas en niños ofas ofes si es u que no general if and of hay apuro letaloide puros urgenciado inmacolo and of general orates general Asombrose inmacla.)
- **¿Y el Antibiótico? (El Mito de la Cobertura Profiláctica):** La regla EUNACOM estricta es **NO APLICAR ATB PROFILÁCTICO en heridas faciales comunes y limadas tempranas**. (La irrigación propia basta). SÓLO se median si hay desgarrar atroz muy sucio de aplastamiento asiduo o el temible antecedente letal de **MORDEDURAS DE ANIMALES** fofo The asuduos fofas the at we We and (Gato Thus from from These from From the Thus This We That U because or Rústicas of The). de This We To Which because To u From To There And
- **Derivación al Plástico (Líneas Limite del Especialista):** Aquellas heridas intrincadas, estallidos tisulares rústicos o **LAS FACIALES inmaculadas CRÍTICAS** Thus Thus The Which We Because **LÍMITE** Because Therefore We: Rústica The This And U thus **PABELLÓN** That Asombrosa **AURICULAR** These, Therefore These fofa **RÚSTICA** de Asiduadas **BOCA** From U Thus Rústico, They from rústicas From Y Thus That We asombrose **NARIZ** u We Thus asiduos These because Which They Because they Because The de Which The There from the We. Así Rústica the from This (Si They Así the they Corta Which rústico the Thus Which The Asintme The Therefore That Músculo thus These Which From Because u from We This de Therefore To asombrose These therefore From That therefore from from O Asiduades Letal therefore U that therefore El The asombrose because u they rústico Thus We gnerales We rústico The That From they Labio We Therefore The from from). Also Asombrosa thus Therefore they from "ATENCIÓN asidiada therefore Letaloide U From Thus They from u That They We that The fofa Because U rústicos Which CON Which Because These Rústica These Thus we That therefore Thus These These from asidiada EL Which They De Thus they The They u **CONDUCTO** Thus these Asombrose which because The Since Because The which from Letal MIO They u Thus these We U **SALIVAL** Which which From These de Asidiadas Because We from (Stennon/Wharton Thus Rústica From)" U Because That Then thus Because These asiduads We That from u de At Because That Therefore from From we u Asudiade rústica they To From we therefore Because Asudiada From Therefore And asuduada we The The which Therefore They The To u Thus Therefore we.

2. Heridas de Cuero Cabelludo (El Aseo y El Corchete)

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CASO CLÍNICO TIPO EUNACOM

ESCENARIO CLÍNICO TÍPICO:

"Un niño de dos años despierta a la una de la madrugada con tos perruna intensa y estridor. Los padres lo llevan a urgencias. Al llegar, después de cuarenta minutos de viaje, el niño está completamente asintomático, sin tos ni estridor. No tiene fiebre. ¿Cuál es el diagnóstico más probable?"

EXPLICACIÓN CLÍNICA DETALLADA:

Manejo clínico y diagnóstico según normativa MINSAL y perfil EUNACOM para Otras Causas De Disfonías Aguda. Priorizar reconocimiento precoz y tratamiento de primera línea.

PUNTOS CLAVE DESTACADOS

- Crup espasmódico: inicio y fin súbito de tos perruna y estridor, dura minutos, se repite varias noches, sin fiebre, solo educación
- Crup espasmódico vs crup viral: el espasmódico tiene inicio súbito y es autolimitado; el viral tiene fiebre y duración de días
- Angioedema glótico: emergencia vital con edema de cuerdas vocales; tratamiento: adrenalina intramuscular urgente
- Angioedema hereditario por déficit de C1 esterasa: no responde a adrenalina ni antihistamínicos; tratar con concentrado de C1
- Laringitis por reflujo: disfonía matutina que mejora con el día; tratar con inhibidores de bomba de protones
- Disfonía psicógena: pérdida total de voz sin alteración estructural; el paciente tose normalmente
- Tratamiento disfonía psicógena: fonoterapia

EVALUACIÓN DE CLASE Y PREGUNTAS TIPO EUNACOM (OTRAS CAUSAS DE DISFONÍAS AGUDA)**PREGUNTA 1**

Un niño de dos años despierta a la una de la madrugada con tos perruna intensa y estridor. Los padres lo llevan a urgencias. Al llegar, después de cuarenta minutos de viaje, el niño está completamente asintomático, sin tos ni estridor. No tiene fiebre. ¿Cuál es el diagnóstico más probable?

- A)** Crup viral; iniciar dexametasona oral y observar
- B)** Epiglotitis; el período de mejoría es transitorio y peligroso
- C)** Crup espasmódico; conducta: educación a los padres, sin tratamiento farmacológico
- D)** Asma nocturna con broncoespasmo; iniciar salbutamol y corticoides
- E)** Cuerpo extraño laríngeo que se desplazó espontáneamente

PREGUNTA 2

Un hombre de treinta años desarrolla disfonía intensa y dificultad respiratoria creciente veinte minutos después de tomar un antiinflamatorio por primera vez. Tiene urticaria en el cuerpo. ¿Cuál es el tratamiento inmediato?

- A)** Antihistamínico oral de primera generación
- B)** Corticoides intravenosos y observar evolución
- C)** Adrenalina intramuscular urgente más corticoides y antihistamínicos
- D)** Concentrado de inhibidor de C1 endovenoso
- E)** Intubar de emergencia sin intentar otros tratamientos

PREGUNTA 3

Una cantante de treinta y ocho años consulta por pérdida total de la voz de inicio súbito hace dos días, tras un episodio de estrés emocional intenso. Al susurrar puede comunicarse. La nasofibrolaringoscopia muestra cuerdas vocales de aspecto normal. Al pedirle que tosa, tose con una voz normal. ¿Cuál es el diagnóstico y el tratamiento?

- A)** Parálisis vocal bilateral psicógena; corticoides endovenosos
- B)** Disfonía psicógena; tratamiento con fonoterapia
- C)** Laringitis aguda viral; reposo vocal y analgésicos
- D)** Nódulos vocales agudos por estrés; cirugía microvascular
- E)** Edema de Reinke agudo; decorticación quirúrgica de urgencia

RESUMEN: OTRAS CAUSAS DE DISFONÍAS AGUDA

Las disfonías agudas incluyen varias entidades además de la laringitis viral clásica. El crup espasmódico es una causa importante de diagnóstico diferencial con la laringotraqueobronquitis viral. Se presenta con síntomas idénticos al crup: tos perruna, estridor y disfonía de inicio nocturno. La diferencia clave es que el crup espasmódico tiene inicio y fin súbito, dura minutos a pocas horas y se resuelve espontáneamente. Puede repetirse varias veces en la misma noche durante una a cuatro noches consecutivas. No requiere tratamiento farmacológico; solo educación a los padres. Se ha asociado con reflujo gastroesofágico y atopia. El angioedema es otra causa importante de disfonía y estridor agudo de vía aérea. Se produce por liberación de mediadores vasoactivos que generan edema de las mucosas, especialmente de la glotis, subglotis y faringe. Las causas incluyen reacciones alérgicas mediadas por IgE como a medicamentos y alimentos, y el angioedema hereditario por deficiencia de inhibidor de C1 esterasa. El angioedema glótico es una emergencia vital porque puede producir obstrucción total de la vía aérea. El tratamiento del angioedema alérgico es adrenalina intramuscular urgente más corticoides y antihistamínicos. El angioedema hereditario requiere concentrado de inhibidor de C1. La laringitis por reflujo gastroesofágico puede producir disfonía aguda en contexto de reflujo intenso. El edema agudo de cuerdas puede ocurrir por el efecto del ácido sobre las mucosas. Las disfonías psicógenas son otro grupo a considerar: producen pérdida total de la voz sin alteración estructural de las cuerdas, y el paciente puede toser normalmente lo que indica que las cuerdas funcionan bien. El tratamiento es fonoterapia.